Spatial Analysis

June 3, 2023

1 Methods

For each sample, we will compare the distribution of each markers in the normal and lesion regions.

- Markers for DD: CD3, CD4, CD8a, CD11c, CD15, CD45, CD68, CD69, CD123, CD279
- Markers for CF: CD1a, CD3, CD4, CD8a, CD11c, CD15, CD45, CD69, CD123, CD279 (CD68 did not pass QC)

For CD123, segmentation is done at flow=0.5 to get more masks in the bright area and a threshold of 5000 is set for total intensity to remove masks in the debris field in the lesion area.

The size of the skin tissue lesion area is determined by morphology using wheat germ agglutinin (WGA) flourescent stain. ImageJ is used to demarcate the lesion area, then a straight line ROI is drawn to span from one side of the lesion to the other side of the lesion, with the endpoints of the line positioned at the outermost skin boundary.

Some considerations on region boundary and distance metric:
1. Should we draw the boundary between normal and lesion based on WGA only or all markers?
2. Shall each ROI be convex?

For (1), the two choices lead are answering different questions. Drawing boundary based on WGA only answers a stronger question than drawing the boundary based on all markers.

For (2), a convex shape is a shape that a line connecting any two points in the shape completely lies within the shape. Practically speaking, in this context it is equivalent to drawing a straight line to separate normal from lesion. Pros for straight boundary: the span between any cell and the closest point on the skin lies completely within the region. So the distance measure is well defined. In contrast, if a line from a cell to its closest point on the skin surface cuts through both normal and lesion regions, the interpretation of distance to skin surface is muddled. Pros for curved boundary: Regions (which is a type of neighborhoods) do not necessarily have geometric shapes.

These two questions are also intertwined. If we use WGA only to draw boundaries, we can only use straight line boundaries because the difference between normal and lesion is only visible in epidermis in WGA staining. A straight line can be drawn in epidermis and extend to dermis, but not a curve.

2 Results

	Counts				Counts/mm2			
	normal		lesion		normal		lesion	
	epi	dermis	epi	dermis	epi	dermis	epi	dermis
CD3	74	1222	158	5395	631	2874	121	1399
CD15	11	238	6567	2867	93	559	5050	743
CD123	131	879	1350	8038	1117	2067	1038	2084
CD11c	343	2056	8029	17905	2926	4836	6174	4643
CD68	154	998	2524	11081	1314	2347	1941	2874
area	0.12	0.43	1.30	3.86				

Table 1: DD

	Counts				Counts/mm2			
	normal		lesion		normal		lesion	
	epi	dermis	epi	dermis	epi	dermis	epi	dermis
CD3	422	8435	331	5629	976	4476	819	5574
CD15	7	2154	756	391	16	1143	1871	387
CD123	76	3680	324	1298	175	1952	801	1285
CD11c	624	8498	1308	4299	1443	4509	3237	4257
area	0.43	1.88	0.40	1.01				

Table 2: CF

	DD	CF	DD/CF
lesion width	2862	1202	2.38
normal width	251	1531	0.16
separation between CD3/CD15 peaks (ref: skin)	607	427	1.42
separation between CD3/CD15 peaks (ref: DEJ)	319	337	0.95
CD3 epi	121	819	0.15
CD3 dermis	1399	5574	0.25
CD3 e/d	0.09	0.15	0.60
CD15 epi	5050	1871	2.70
CD15 dermis	743	387	1.92
CD15 e/d	6.80	4.83	1.41
CD123 epi	1038	801	1.30
CD123 dermis	2084	1285	1.62
CD123 e/d	0.50	0.62	0.81
CD11c epi	6174	3237	1.91
CD11c dermis	4643	4257	1.09
CD11c e/d	1.33	0.76	1.75

Table 3: All numbers are for the lesion except the second row. The cell counts are per mm2. The widths and seperation are measured in um.

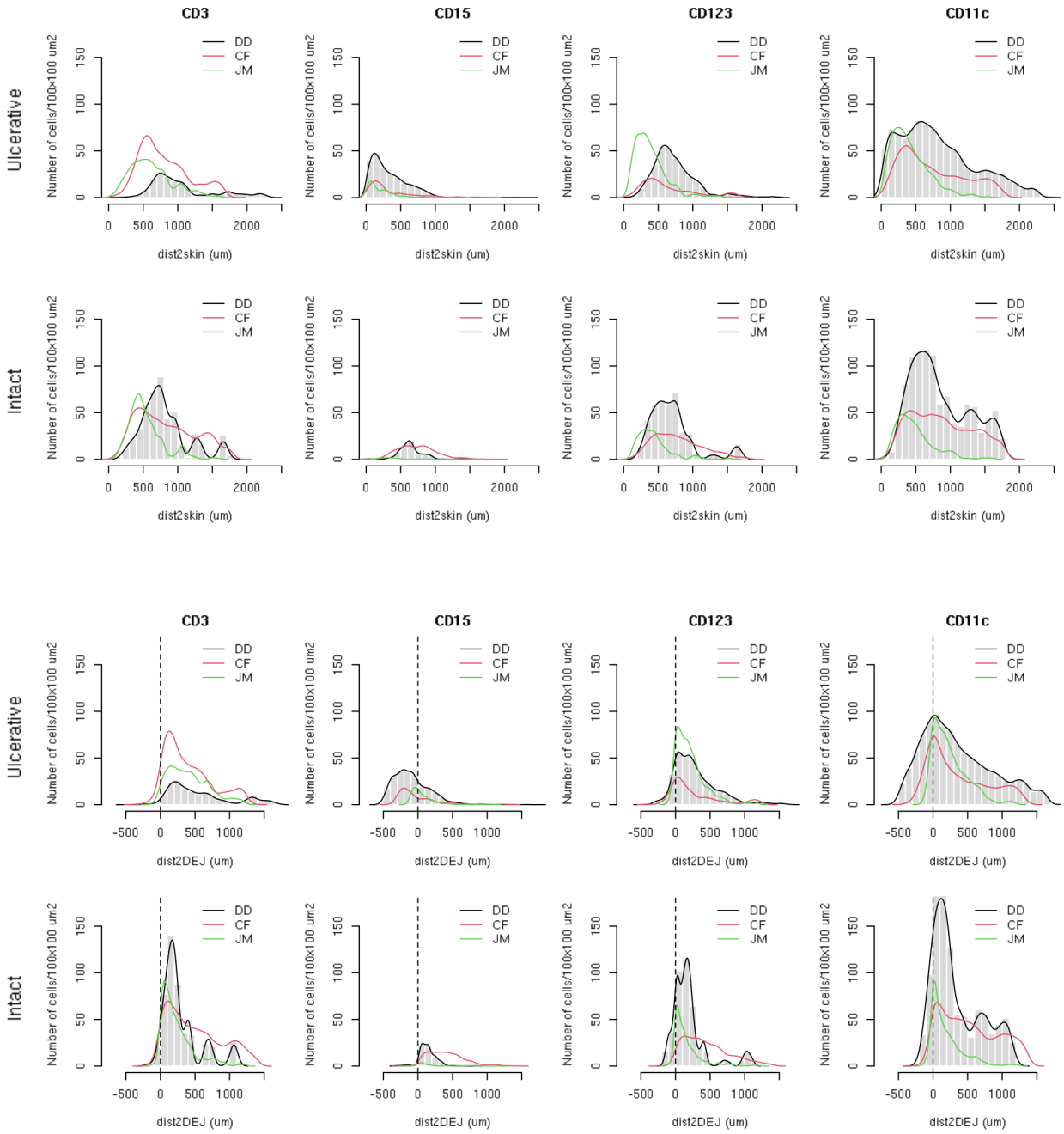


Figure 1: Number of cells per 100x100 um2 tissue as a function of distance to DEJ or skin. The curves are smoothed over histograms and each bin in the histogram represents a 100 um interval. Adding up the counts from all the bins gives the number of cells in tissue of 100 um in width. For example, CD123 epi count 1038/2862*100=36.

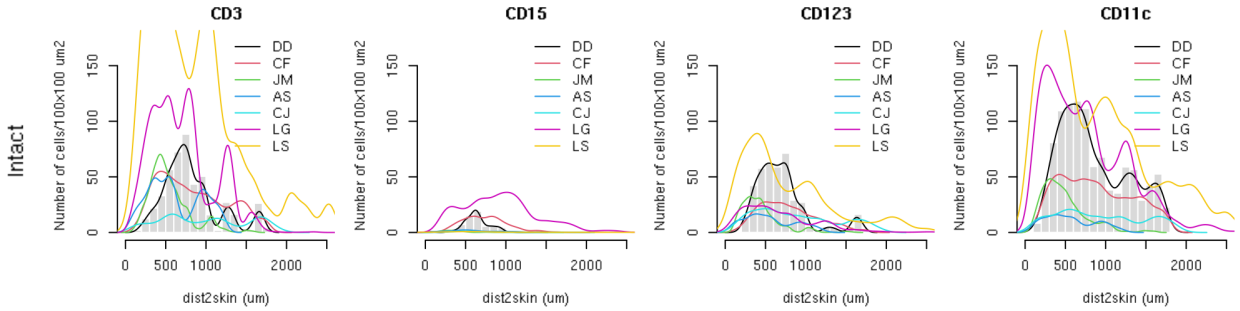


Figure 2: Number of cells per 100x100 um2 tissue as a function of distance to DEJ or skin. The curves are smoothed over histograms and each bin in the histogram represents a 100 um interval. Adding up the counts from all the bins gives the number of cells in tissue of 100 um in width.

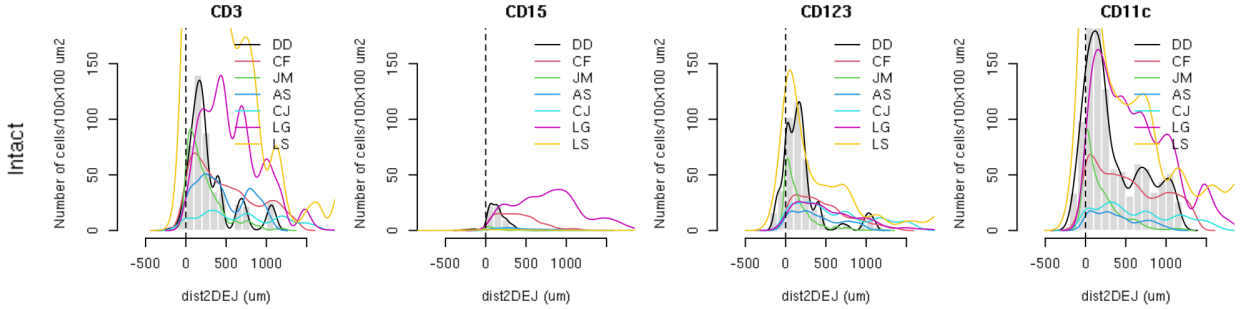


Figure 3: Number of cells per 100x100 um2 tissue as a function of distance to DEJ or skin. The curves are smoothed over histograms and each bin in the histogram represents a 100 um interval. Adding up the counts from all the bins gives the number of cells in tissue of 100 um in width.

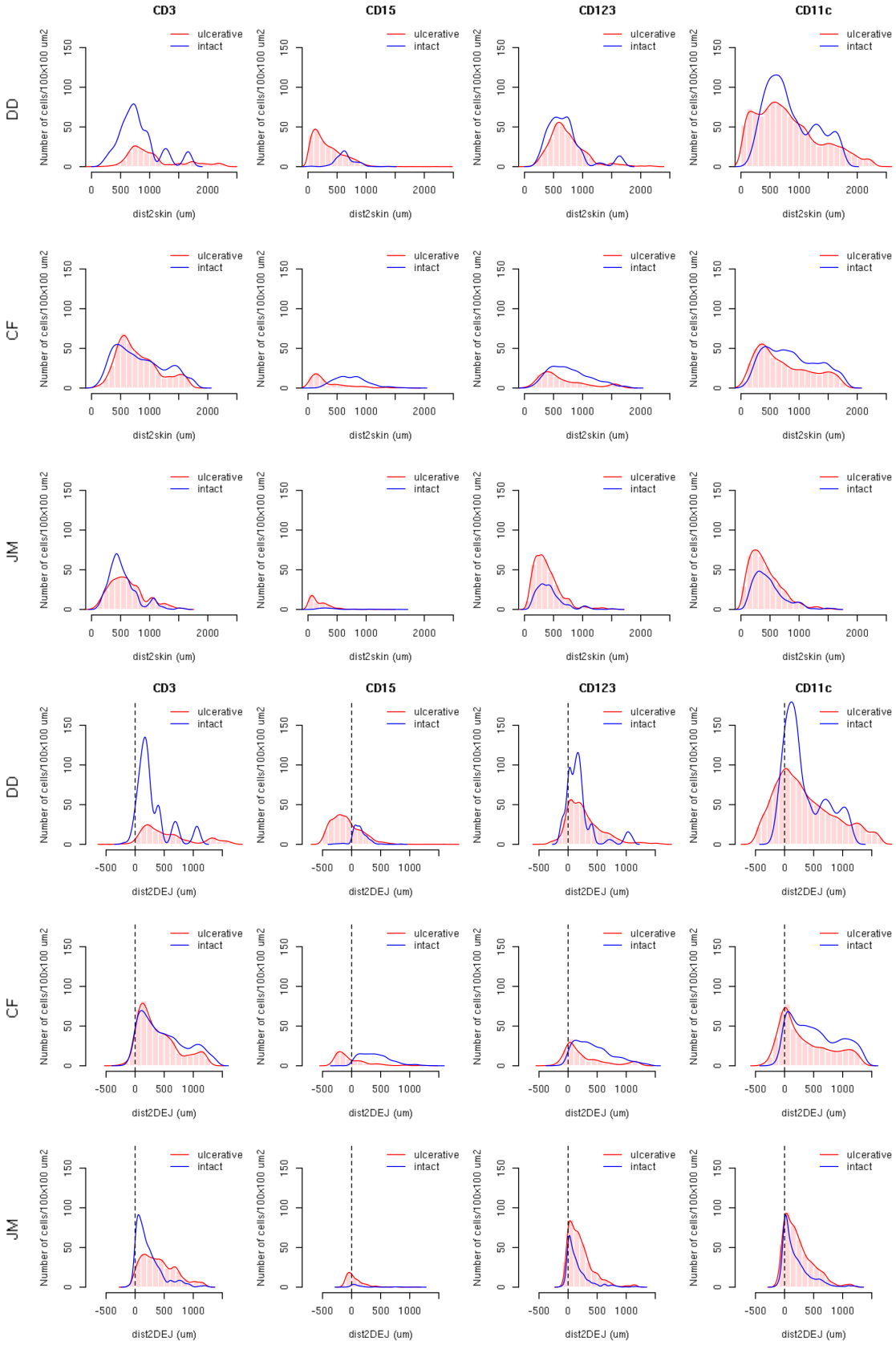


Figure 4: Number of cells per 100x100 um2 tissue as a function of distance to skin or DEJ. The curves are smoothed over histograms and each bin in the histogram represents a 100 um interval. Adding up the counts from all the bins gives the number of cells in tissue of 100 um in width.

Comparing the first two rows in the first column, we see that the ratio of the area under the curve across both epi and dermis between red and blue(ulcerative and intact) is about 1 for CD3/CF and 1/3 for CD3/DD. This suggests that CD3 is depleted in lesion in DD, but not in CF.

Compare lesion with normal in CD15/CF, we see that the lesion peak is shifted to the left compared to the normal peak. This pattern is more clear in CF than in DD. We don't see this pattern for CD3.

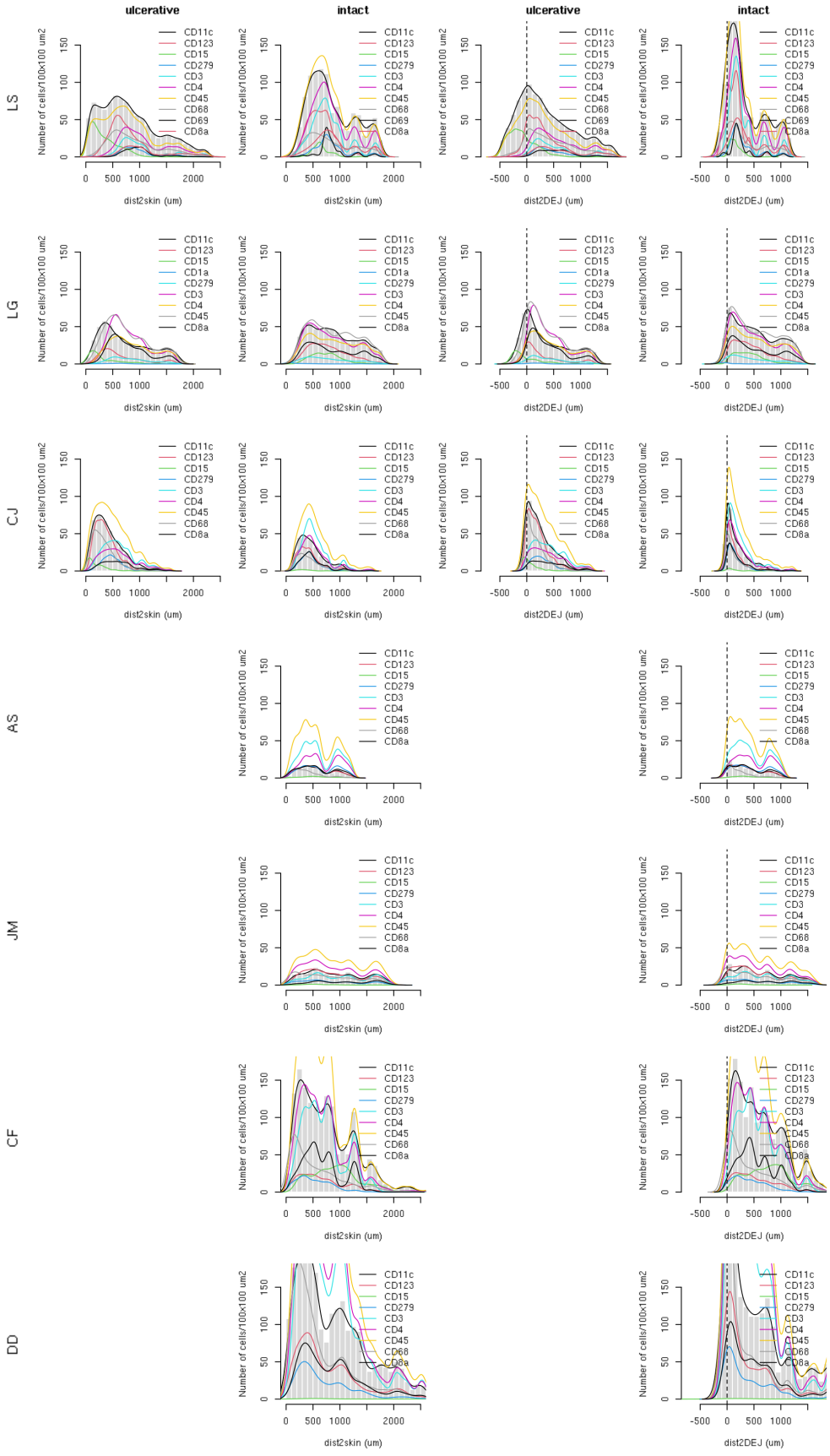
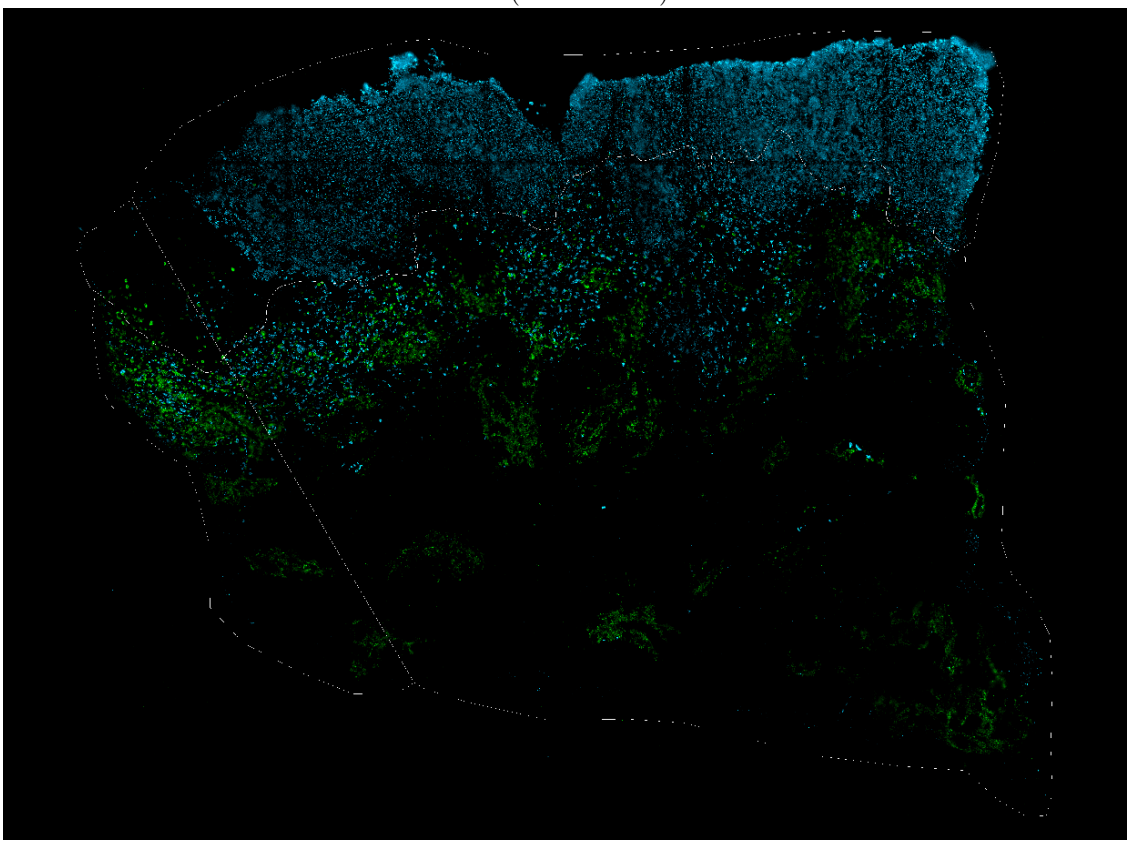


Figure 5: Number of cells per 100x100 um2 tissue as a function of distance to skin. The curves are smoothed over histograms and each bin in the histogram represents a 100 um interval. Adding up the counts from all the bins gives the number of cells in tissue of 100 um in width.

That the CD15 dist2skin peak in DD/lesion is near skin while the corresponding dist2DEJ peak is near DEJ is due to the fact that the DEJ line is very curvy.

DD (6540 × 5262)



CF (5296 × 4040)

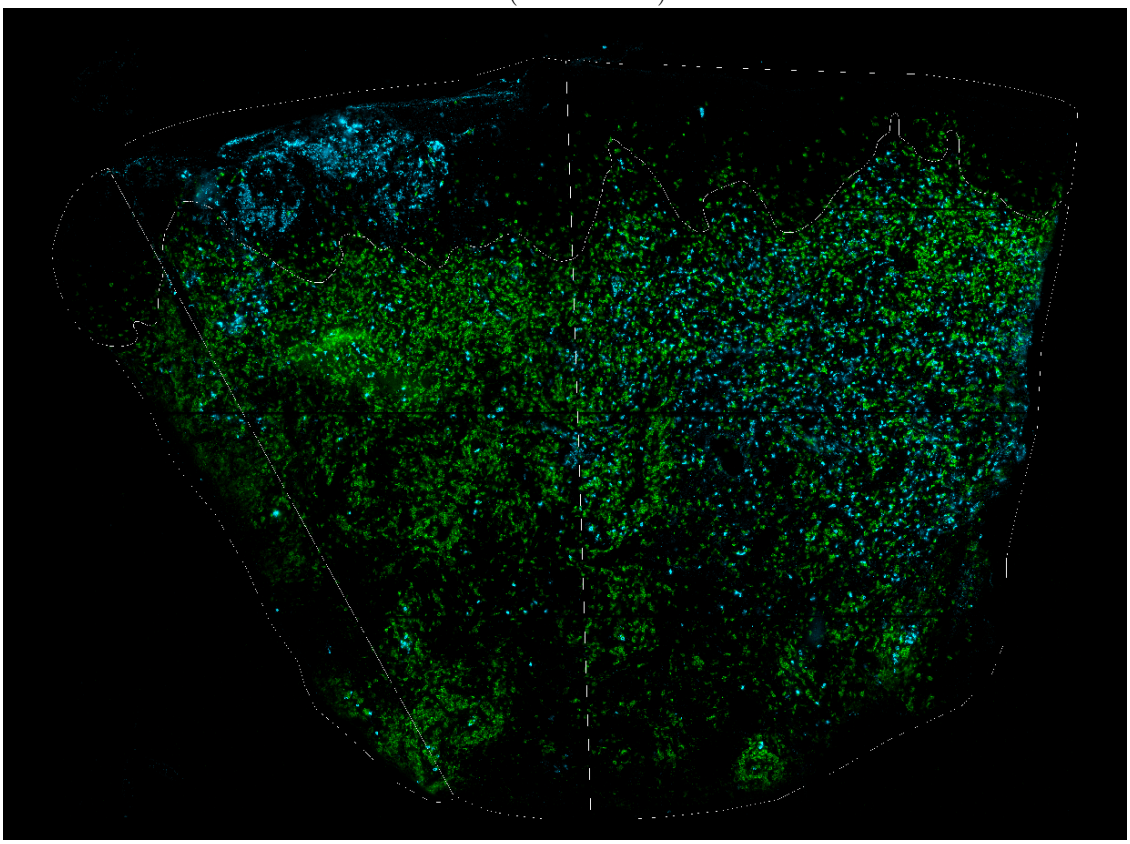


Figure 6: CD3: green, contrast limits 0-0.5; CD15: cyan. The intact/ulcerative boundary and the epidmeris/dermis boundary are shown in white. Outer boundaries are not tight.

In Figure 6 we see that DD lesion has less CD3 and is further away from DEJ than CF. Correspondingly, in Figure 1, we see that in the dermis region close to DEJ, the count density is about 50 for CF/ulcerative, CF/intact, and CF/ulcerative; but for DD/ulcerative, the count density is much lower.

3 Discussion

Neutraphil to lymphocyte ratio (NLR) in blood is a prognostic factor for cancer. Lower is better.

4 References

Chan A, Bentzen S, Rout A, Miller K. Predicting if lung cancer will relapse-the role of neutrophil/lymphocyte ratio. Hematol Oncol Stem Cell Ther. 2023;16(2):158–61.

Costa T, Nogueiro J, Ribeiro D, Viegas P, Santos-Sousa H. Impact of serum albumin concentration and neutrophil-lymphocyte ratio score on gastric cancer prognosis. Langenbecks Arch Surg. 2023;408(1):57.

Chen W, Xin S, Xu B. Value research of NLR, PLR, and RDW in prognostic assessment of patients with colorectal cancer. J Healthcare Eng. 2022;16(2022):7971415.